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PACK INSERT FOR MALAYSIA

Dormicum® Tablets

Midazolam

1. DESCRIPTION

1.1 Therapeutic/ Pharmacologic Class of Drug
Dormicum Tablet is a sleep-inducing agent belonging to the benzodiazepines.

1.2 Type of Dosage Form
Tablets.

1.3 Route of Administration
Oral use.

1.4 Qualitative and Quantitative Composition
Active ingredient: midazolam as the maleate.
Tablets containing midazolam maleate equivalent to 7.5 mg of midazolam.
The 7.5 mg tablet is oval, cylindrical, biconvex and of a white to almost white colour.
Excipients: described as per local requirements (Dormicum tablets contain anhydrous lactose. For warning related to lactose monohydrate, see section 2.4.1).

2. CLINICAL PARTICULARS

2.1 Therapeutic Indications
Disturbances of sleep rhythm and all forms of insomnia, particularly difficulty in falling asleep either initially or after premature awakening. Sedation in premedication before surgical or diagnostic procedures.

2.2 Dosage and Administration
Duration of treatment should be as short as possible. Generally, the duration of treatment varies from a few days to a maximum of 2 weeks. The tapering-off process should be tailored to the individual. Treatment with Dormicum should not be terminated abruptly (see section 2.4.2).
In certain cases, extension beyond the maximum treatment period may be necessary; if so, it should not take place without reevaluation of the patient's status. Owing to the rapid onset of action, Dormicum tablets should be taken immediately before going to sleep, and swallowed whole with fluid. Dormicum can be taken at any time of the day, provided the patient is subsequently assured of at least 7-8 hours undisturbed sleep.

Standard dosage
Dosage range: 7.5-15 mg.
Treatment should be started with the lowest recommended dose. The maximum dose should not be exceeded because of the increased risk of CNS adverse effects possibly including clinically relevant respiratory and cardiovascular depression.

Premedication
In premedication, Dormicum should be given 30-60 minutes before the procedure.

2.2.1 Special Dosage Instructions
Elderly and/or debilitated patients
In elderly and/or debilitated patients the recommended dose is 7.5 mg.
Elderly patients showed a larger sedative effect; therefore they may be at increased risk of cardio-respiratory depression as well. Thus, Dormicum should be used very carefully in elderly patients, and if needed, a lower dose should be considered.

Patients with hepatic impairment
Patients with severe hepatic impairment should not be treated with Dormicum (see section 2.3). In patients with mild to moderate hepatic impairment, the lowest dose possible should be considered, not exceeding 7.5 mg (see section 3.2.5).

Patients with renal impairment
In patients with severe renal impairment, Dormicum may be accompanied by more pronounced and prolonged sedation, possibly including clinically relevant respiratory and cardiovascular depression. Dormicum should therefore be dosed carefully in this patient population and titrated for the desired effect. The lowest dose should be considered, not exceeding 7.5 mg (see section 3.2.5).

2.3 Contraindications
Dormicum must not be used in patients with:
- Severe respiratory insufficiency;
- Severe hepatic impairment (benzodiazepines are not indicated to treat patients with severe hepatic impairment as they may cause encephalopathy);
- Sleep apnoea syndrome;
- Known hypersensitivity to benzodiazepines or to any of their formulation excipients;
- Myasthenia gravis.

Dormicum tablets should not be given to children, 12 years of age and under, because the available strengths of tablets do not allow for appropriate dosing in this patient population.
Dormicum tablets should not be given to patients receiving concomitant therapy with very strong CYP3A inducers or inhibitors (ketoconazole, itraconazole, voriconazole, HIV protease inhibitors including ritonavir-boosted formulations,) and the HCV protease inhibitors boceprevir and telaprevir (see section 2.4.4).

2.4 Warnings and Precautions

2.4.1 General
Information should be given to the patients about following warnings and precautions:

Tolerance
Some loss of efficacy to the hypnotic effects of short-acting benzodiazepines may develop after repeated use for a few weeks.

Duration of treatment
The duration of treatment with benzodiazepine hypnotics should be as short as possible (see section 2.2), and it should not exceed 2 weeks. The tapering-off process should be tailored to the individual. Extension beyond this period should not take place without reevaluation of the situation.

Rebound insomnia
When discontinuing Dormicum therapy, insomnia may reoccur, possibly with a higher severity than before starting treatment ("rebound insomnia"). Rebound insomnia, a transient syndrome, may be accompanied by other reactions including mood changes, anxiety and restlessness. The risk of rebound phenomena is greater after abrupt discontinuation of treatment. Therefore, it is recommended that the dosage of Dormicum is decreased gradually (see section 2.4.2).

Amnesia
Dormicum may cause anterograde amnesia, which occurs most frequently within the first few hours after ingesting the product. In order to reduce the risk, patients should ensure that they are able to have an uninterrupted sleep of 7-8 hours (see also section 2.6).

Residual effects
Provided the oral dose of Dormicum is not larger than 15 mg/day and the patient is assured of at least 7 to 8 hours undisturbed sleep, no residual effect is observed following oral administration of Dormicum tablet in standard patients as confirmed by clinical observations using sensitive pharmacological methods.

Psychiatric and 'paradoxical' reactions
Paradoxical reactions such as restlessness, agitation, irritability, aggressiveness, anxiety, and more rarely, delusion, anger, nightmares, hallucinations, psychosis, inappropriate behavior and other adverse behavioral effects are known to occur when using benzodiazepines. Should this be so, use of the drug should be discontinued.
These effects are more likely to occur in the elderly.

Specific patient groups
In elderly and/or debilitated patients, as well as in patients with respiratory or cardiovascular impairment, the recommended dose is 7.5 mg. These patients may be more sensitive to the clinical side effects of midazolam like cardio-respiratory depression. Thus, Dormicum should be used very carefully in these patient populations and if needed a lower dose should be considered (see section 2.2.1).

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Dosage instructions for patients with hepatic and/or renal impairment are described in section 2.2.1.
Benzodiazepines are not recommended for the primary treatment of psychotic illness. Benzodiazepines should not be used alone to treat depression or anxiety associated with depression as suicide may occur in such patients.

Concomitant use of alcohol/ CNS depressants
The concomitant use of Dormicum with alcohol or/and CNS depressants should be avoided. Such concomitant use has the potential to increase the clinical effects of Dormicum possibly including severe sedation that could result in coma or death, clinically relevant respiratory and/ or cardio-vascular depression (see section 2.4.4).

Medical history of alcohol or drug abuse
Dormicum should be avoided in patients with a medical history of alcohol or drug abuse.

Co-medication with drugs that alter CYP3A activity
Midazolam pharmacokinetics is altered in patients receiving concomitantly compounds that inhibit or induce CYP3A. Consequently, the clinical and adverse effects may be increased or decreased respectively (see section 2.4.4).

Lactose intolerance
Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.
Anaphylaxis (severe allergic reaction) and angioedema (severe facial swelling) which can occur as early as the first time the product is taken.
Complex sleep-related behaviours which may include sleep driving, making phone calls, preparing and eating food (while asleep).

Risks from Concomitant Use with Opioids
Profound sedation, respiratory depression, coma, and death may result from the concomitant use of Dormicum with opioids. Observational studies have demonstrated that concomitant use of opioids and benzodiazepines increases the risk of drug-related mortality compared to use of opioids alone. Because of these risks, reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate.
If the decision is made to newly prescribe a benzodiazepine and an opioid together, prescribe the lowest effective dosages and minimum durations of concomitant use.
If the decision is made to prescribe a benzodiazepine in a patient already receiving an opioid, prescribe a lower initial dose of the benzodiazepine than indicated in the absence of an opioid, and titrate based on clinical response.
If the decision is made to prescribe an opioid in a patient already taking a benzodiazepine, prescribe a lower initial dose of the opioid, and titrate based on clinical response.
Follow patients closely for signs and symptoms of respiratory depression and sedation. Advise both patients and caregivers about the risks of respiratory depression and sedation when Dormicum is used with opioids. Advise patients not to drive or operate heavy machinery until the effects of concomitant use of the opioid have been determined. Screen patients for risk of substance use disorders, including opioid abuse and misuse, and warn them of the risk for overdose and death associated with the use of opioids (see section 2.4.4).

2.4.2 Drug Abuse and Dependence

Dependence
Use of Dormicum may lead to the development of physical and psychological dependence. The risk of dependence increases with dose and duration of treatment; it is also greater in patients with a medical history of alcohol and/ or drug abuse.

Withdrawal
Withdrawal symptoms may consist of headaches, diarrhoea, muscle pain, extreme anxiety, tension, restlessness, confusion and irritability. In severe cases, the following symptoms may occur: derealisation, depersonalization, hyperacusis, numbness and tingling of the extremities, hypersensitivity to light, noise and physical contact, hallucinations or convulsions.
Since the risk of withdrawal phenomena/ rebound insomnia is higher after abrupt discontinuation of treatment, it is recommended that the dosage be decreased gradually (see sections 2.2 and 2.4.1)).

2.4.3 Ability to Drive and Use Machines

Sedation, amnesia, impaired concentration and impaired muscular function adversely affect the ability to drive or to use machines. Prior to receiving Dormicum, the patient should be warned not to drive a vehicle or operate a machine until completely recovered. The physician should decide when these activities may be resumed.
If sleep duration is insufficient or alcohol is consumed, the likelihood of impaired alertness may be increased (see section 2.4.4).

2.4.4 Interactions with other Medicinal Products and other Forms of Interaction

Pharmacokinetic Drug-Drug Interaction (DDI)
(see sections 2.3 and 2.4.1)
Midazolam is almost exclusively metabolised by cytochrome P450 3A (CYP3A4 and CYP3A5). Inhibitors and inducers of CYP3A have the potential to increase and decrease the plasma concentrations and, subsequently, the pharmacodynamic effects of midazolam. No other mechanism than modulation of CYP3A activity has been proven as a source for a clinically relevant pharmacokinetic drug-drug interaction with midazolam. Midazolam is not known to change the pharmacokinetics of other drugs.
When co-administered with a CYP3A inhibitor, the clinical effects of oral midazolam may be stronger and also longer lasting and a lower dose may be required. Conversely the effect of midazolam may be weaker and last shorter when co-administered with a CYP3A inducer and a higher dose may be required.
In case of CYP3A induction and irreversible inhibition (so-called mechanism-based inhibition), the effect on the pharmacokinetics of midazolam may persist for several days up to several weeks after administration of the CYP3A modulator. Examples of mechanism-based CYP3A inhibitors include antibacterial drugs (e.g. clarithromycin, erythromycin, isoniazid), antiretrovirals (e.g. HIV protease inhibitors such as ritonavir, including ritonavir-boosted protease inhibitors; delavirdine), calcium channel blockers (e.g. verapamil, diltiazem), tyrosine kinase inhibitors (e.g. imatinib, lapatinib, idelalisib) or the oestrogen receptor modulator raloxifene.
Ethinyloestradiol combined with norgestrel or gestodene did not modify exposure to midazolam to a clinically significant degree.

Drugs that inhibit CYP3A4
Classification of CYP3A inhibitors
CYP3A inhibitors can be classified according to the strength of their inhibitory effect and to the importance of the clinical modifications when they are administered concomitantly with oral midazolam:

- **Very strong inhibitors:** Midazolam AUC increased > 10-fold. The following drugs fall into this category: e.g. ketoconazole, itraconazole, voriconazole, HIV protease inhibitors including ritonavir boosted protease inhibitors:
  Combination of midazolam administered orally with very strong CYP3A inhibitors is contraindicated (see section 2.3***).***
- **Strong inhibitors:** Midazolam AUC increased by 5 to 10-fold. The following drugs fall into this category: e.g. high dose clarithromycin, tyrosine kinase inhibitors (such as idelalisib) and the HCV protease inhibitors boceprevir and telaprevir.
  Concomitant administration of oral midazolam and boceprevir and telaprevir is contraindicated (see section 2.3).
- **Moderate inhibitors:** Midazolam AUC increased by 2 to 5-fold. The following drugs fall into this category: e.g. fluconazole, telithromycin, erythromycin, diltiazem, verapamil, nefazodone, NK1 receptor antagonists (aprepitant, netupitant, casopitant), tabimoreline, posaconazole.
  Patients receiving midazolam with strong or moderate CYP3A inhibitors require careful evaluation because the side effects of midazolam may be potentiated (see section 2.4.1).
- **Weak inhibitors:** Midazolam AUC increased by 1.25 to < 2-fold. The following drugs and herbals fall into this category: e.g. fentanyl, roxithromycin, cimetidine, ranitidine, fluvoxamine, bicalutamide,

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propiverine, everolimus, cyclosporine, simeprevir, grapefruit juice, *Echinacea purpurea*, berberine as also contained in goldenseal.
Concomitant administration of midazolam with weak CYP3A inhibitors does not usually lead to a relevant change of midazolam clinical effect.

Drugs that induce CYP3A
Patients receiving a combination of midazolam with CYP3A inducers may require a higher midazolam dose in particular if midazolam is co-administered with strong CYP3A inducers. Strong CYP3A inducers (≥ 80% decrease in AUC) include: e.g. rifampin, carbamazepine, phenytoin, enzalutamide and mitotane with its long lasting CYP3A4-inducing effect, while moderate CYP3A inducers (50-80% decrease in AUC) include St John's wort and weak inducers (20-50% decrease in AUC) include efavirenz, clobazam, ticagrelor, vemurafenib, quercetin and *Panax ginseng*.

Pharmacodynamic Drug-Drug Interactions (DDI)
The co-administration of midazolam with other sedative/ hypnotic agents, including alcohol, is likely to result in increased sedative/ hypnotic effects. Examples include opiates/ opioids (when they are used as analgesics, antitussives or substitutive treatments), antipsychotics, other benzodiazepines used as anxiolytics or hypnotics, barbiturates, propofol, ketamine, etomidate; sedative antidepressants, antihistamines and centrally acting antihypertensive drugs. Midazolam decreases the minimum alveolar concentration (MAC) of inhalational anaesthetics. Enhanced side effects such as sedation and cardio-respiratory depression may also occur when midazolam is co-administered with any centrally acting depressants including alcohol. Alcohol should be avoided in patients receiving midazolam (see sections 2.4. and 2.7 for warning of other central nervous system depressants, including alcohol).
Drugs increasing alertness/ memory like the AChE inhibitor physostigmine reversed the hypnotic effects of midazolam. Similarly, 250 mg of caffeine partly reversed the sedative effect of midazolam.

Opioids
Due to additive pharmacologic effect, the concomitant use of opioids with benzodiazepines increases the risk of respiratory depression, profound sedation, coma and death. The concomitant use of opioids and benzodiazepines increases the risk of respiratory depression because of actions at different receptor sites in the central nervous system that control respiration. Opioids interact primarily at μ-receptors, and benzodiazepines interact at $GABA_A$ sites. When opioids and benzodiazepines are combined, the potential for benzodiazepines to significantly worsen opioid-related respiratory depression exists.
Reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate (see section 2.4).
Limit dosage and duration of concomitant use of benzodiazepines and opioids, and follow patients closely for respiratory depression and sedation.

2.5 Use in Special Populations

2.5.1 Pregnancy
Benzodiazepines should be avoided during pregnancy unless there is no safer alternative.
An increased risk of congenital malformation associated with the use of benzodiazepines during the first trimester of pregnancy has been suggested. If the product is prescribed to a woman of childbearing potential, she should contact her physician regarding discontinuation of the product if she intends to become or suspects that she is pregnant.
The administration of midazolam in the last trimester of pregnancy or at high doses during labour has been reported to produce irregularities in the foetal heart rate, hypotonia, poor sucking and hypothermia and moderate respiratory depression in the neonate. Moreover, infants born to mothers who took benzodiazepines chronically during the latter stages of pregnancy may have developed physical dependence and may be at some risk of developing withdrawal symptoms in the postnatal period.

2.5.2 Labor and Delivery
See section 2.5.1 Pregnancy.

2.5.3 Nursing Mothers
Since midazolam passes into breast milk, Dormicum should not be administered to breast-feeding mothers.

2.5.4 Paediatric Use
See section 2.3.

2.5.5 Geriatric Use
See sections 2.2.1 and 2.4.1.

2.5.6 Renal Impairment
There is a greater likelihood of adverse drug reactions in patients with severe kidney disease. (see sections 2.2.1 and 3.2.5).

2.5.7 Hepatic Impairment
See sections 2.2.1 and 2.3.

2.6 Undesirable Effects

2.6.1 Post Marketing
Immune System Disorders: Hypersensitivity reactions and angioedema may occur in susceptible individuals.

Psychiatric Disorders: Confusional state, disorientation, emotional and mood disturbances. These phenomena occur predominantly at the start of therapy and usually disappear with repeated administration. Changes in libido have been reported occasionally.
Depression: Pre-existing depression may be unmasked during benzodiazepine use.
Paradoxical reactions such as restlessness, agitation, hyperactivity, nervousness, anxiety, irritability, aggressiveness, anger, nightmares, abnormal dreams, hallucinations, inappropriate behaviour and other adverse behavioural effects are known to occur. Should this be the case, the use of the drug should be discontinued. These effects are more likely to occur in the elderly.
Dependence: Use (even at therapeutic doses) may lead to the development of physical dependence. Abrupt discontinuation of the therapy may result in withdrawal or rebound phenomena including rebound insomnia, mood changes, anxiety and restlessness (see section 2.4.1).
Psychological drug dependence may occur. Abuse has been reported in poly-drug abusers.

Nervous System Disorders: Drowsiness during the day, headache, dizziness, decreased alertness, ataxia. These phenomena occur predominantly at the start of therapy and usually disappear with repeated administration.
When used as premedication, this product may contribute to postoperative sedation.
Anterograde amnesia may occur with therapeutic doses, the risk increasing at higher dosages. Amnestic effects may be associated with inappropriate behaviour (see section 2.4.1).

Eye Disorders: Diplopia, this phenomenon occurs predominantly at the start of therapy and usually disappears with repeated administration.

Gastrointestinal Disorders: Gastrointestinal disturbances, have been reported occasionally.

Skin and Subcutaneous Tissue Disorders: Skin reactions have been reported occasionally.

Musculoskeletal and Connective Tissue Disorders: Muscle weakness, this phenomenon occurs predominantly at the start of therapy and usually disappears with repeated administration.

General Disorders and Administration Site Conditions: Fatigue, this phenomenon occurs predominantly at the start of therapy and usually disappears with repeated administration.

Injury, Poisoning and Procedural Complications: There have been reports of falls and fractures in benzodiazepine users. The risk is increased in those taking concomitant sedatives (including alcoholic beverages) and in the elderly.

Respiratory Disorders: Respiratory depression was reported.

Cardiac Disorders: Cardiac failure including cardiac arrest was reported.

2.7 Overdose
Symptoms
Benzodiazepines commonly cause drowsiness, ataxia, dysarthria and nystagmus. Overdose of Dormicum is seldom life-threatening, if the drug is taken alone, but may lead to areflexia, apnoea, hypotonia, hypotension, cardiorespiratory depression and rare cases to coma. Coma, if it occurs, usually lasts a few hours but it may be more protracted and cyclical, particularly in elderly patients.

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Benzodiazepine respiratory depressant effects are more serious in patients with respiratory disease.
Benzodiazepines increase the effects of other central nervous system depressants, including alcohol.

Treatment
Monitor the patient's vital signs and institute supportive measures as indicated by the patient's clinical state. In particular, patients may require symptomatic treatment for cardiorespiratory effects or central nervous system effects.
If taken orally further absorption should be prevented using an appropriate method e.g. treatment within 1-2 hours with activated charcoal. If activated charcoal is used airway protection is imperative for drowsy patients. In case of mixed ingestion gastric lavage may be considered, however not as a routine measure.
If CNS depression is severe consider the use of flumazenil (Anexate®), a benzodiazepine antagonist. This should only be administered under closely monitored conditions. It has a short half-life (about an hour), therefore patients administered flumazenil will require monitoring after its effects have worn off. Flumazenil is to be used with extreme caution in the presence of drugs that reduce seizure threshold (e.g. tricyclic antidepressants). Refer to the prescribing information for flumazenil (Anexate®), for further information on the correct use of this drug.

3. PHARMACOLOGICAL PROPERTIES AND EFFECTS

3.1 Pharmacodynamic Properties

3.1.1 Mechanism of Action

Dormicum has a hypnotic and sedative effect characterised by a rapid onset and short duration of action. It also exerts anxiolytic, anticonvulsant and muscle-relaxant effects. Dormicum impairs psychomotor function after single and/or multiple doses but causes minimal haemodynamic changes.
The central actions of benzodiazepines are mediated through an enhancement of the GABAergic neurotransmission at inhibitory synapses. In the presence of benzodiazepines, the affinity of the GABA receptor for the neurotransmitter is enhanced through positive allosteric modulation resulting in an increased action of released GABA on the postsynaptic transmembrane chloride ion flux.

3.2 Pharmacokinetic Properties

3.2.1 Absorption

Midazolam is absorbed rapidly and completely after oral administration.
Due to the substantial first-pass effect, the absolute bioavailability of oral midazolam ranges 30-70%.
Midazolam exhibits linear pharmacokinetics following oral doses of 7.5-20 mg.
After a single administration of a Dormicum 15 mg tablet, maximum plasma concentrations of 70-120 ng/ml are reached within one hour. Food prolongs the time to peak plasma concentration by around one hour, pointing to a reduced absorption rate of midazolam. The absorption half-life is 5-20 minutes.

3.2.2 Distribution

The tissue distribution of midazolam is very rapid and in most cases a distribution phase is not apparent or is essentially completed within 1-2 hours after oral administration. The volume of distribution at steady state is 0.7-1.2 l/kg. 96-98% of midazolam is bound to plasma proteins. The major fraction of plasma protein binding is due to albumin. There is a slow and insignificant passage of midazolam into cerebrospinal fluid. In humans, midazolam has been shown to cross the placenta slowly and to enter fetal circulation. Small quantities of midazolam are found in human milk. Midazolam is not a substrate for drug transporters.

3.2.3 Metabolism

Midazolam is almost entirely eliminated by biotransformation. Midazolam is hydroxylated by Cytochrome P450, CYP3A isozymes. Both isozymes, CYP3A4 and also CYP3A5 are actively involved in the two key pathways for the hepatic oxidative metabolism of midazolam. The metabolism of midazolam after oral administration relies to a comparable extent on intestinal CYP3A and on hepatic CYP3A.
There are two main oxidised metabolites 1'-hydroxymidazolam (also named α-hydroxymidazolam) and 4-hydroxymidazolam. 1'-hydroxymidazolam is the major urinary and plasma metabolite. Plasma concentrations of 1'-hydroxymidazolam may reach 30-50% those of the parent compound. 1'-hydroxymidazolam is pharmacologically active and contributes significantly (about 34%) to the effects of oral midazolam.

3.2.4 Elimination

In young healthy volunteers, the elimination half-life of midazolam ranges from 1.5 to 2.5 hours. The elimination half-life of 1'-hydroxymidazolam is shorter than 1 hour; therefore, after midazolam administration the concentration of the parent compound and the main metabolite decline in parallel. Less than 1% of the dose is recovered in urine as unchanged drug. 60-80% of the dose is glucuronidated and excreted in the urine in the form of 1'-hydroxymidazolam conjugate. Midazolam is a non-accumulating drug when given once daily. Repeated administrations of midazolam do not induce drug-metabolising enzymes.

3.2.5 Pharmacokinetics in Special Populations

Elderly
In elderly male subjects over 60 years of age, the elimination half-life of midazolam was significantly prolonged by a factor 2.5 as compared with younger male subjects. Total midazolam clearance was significantly reduced in male elderly subjects and the bioavailability of the oral tablet was significantly increased. However no significant differences were observed in elderly female compared to younger subjects.

Patients with hepatic impairment
The pharmacokinetics of midazolam were significantly modified in patients with chronic liver disease including advanced liver cirrhosis. In particular, as a consequence of a decreased liver clearance, the elimination half-life was prolonged and the absolute bioavailability of oral midazolam was significantly increased in cirrhotic patients compared to control.

Patients with renal impairment
The pharmacokinetics of unbound midazolam are not altered in patients with severe renal impairment. The pharmacologically mildly active major midazolam metabolite, 1'-hydroxymidazolam glucuronide, which is excreted through the kidney, accumulates in patients with severe renal impairment. This accumulation produces a prolonged sedation. Oral midazolam should therefore be administered carefully and titrated to the desired effect (see section 2.2.1).

Obese patients
In obese patients, the volume of distribution of midazolam is increased. As a consequence, the mean elimination half-life of midazolam is longer in obese than in non-obese patients (5.9 hours vs 2.3 hours). The oral bioavailability of the midazolam tablet was not different in obese patients compared to non-obese patients.

4. PHARMACEUTICAL PARTICULARS

4.1 Storage

This medicine should not be used after the expiry date (EXP) shown on the pack. See also outer pack for storage remark.

4.2 Packs

Tablets 7.5 mg (white) 100

Medicine: Keep out of reach of children

Revision Date: November 2020

Made for
CHEPLAPHARM Arzneimittel GmbH,
Greifswald, Germany
by Recipharm Leganés S.L.U.
Leganés, Spain